

Guidelines for Prior Approval of Waivered Services

- **Prior approval of authorizations is required for all CLTC programs**
 - Community Choice Waiver
 - HIV Waiver
 - Ventilator Waiver
 - Medically Complex Children's Waiver
 - Children's PCA
 - Children's PDN
- **When creating a new service plan, the assessor needs to indicate the requested service level for EACH service identified in the service plan.**
- **When reviewing a new service plan, the reviewer needs to enter an approval level for each service prior to signing off of the service plan.**
- Waivered services should not be requested until all other community resources have been explored and documented.
- The case manager must critically examine a participant's needs.
- The case manager needs to explore a case mix of services to meet the participant's need.

Examples

Current Service Level	Case Mix of Services
PC2 for 20 hours	PC2 for 10 hours and PC1 for 10 hours
PC2 for 15 hours and PC1 for 5 hours	PC2 for 10 hours and PC1 for 10 hours
PC2 for 25 hours	PC 2 for 5 hours and ADHC for 5 days

- Only services and service levels the case manger thinks are necessary should be requested in the approval process.
- If the participant insists on receiving a service that the case manager thinks is not necessary, current policy should be followed. This requested service should not show up on the prior approval section, since there should not be an intervention to trigger it.
- The reviewer needs to review the assessment and service plan to determine participant's level of assistance required. This should be compared to the justification for services on the screen.
- The reviewer will give approval of service levels based on this information and discussion with the case manager.

Prior Approval Points related to Phoenix

- For service plans that were created prior to October 15, 2010, Phoenix auto generated a service request level and approval level based on what was currently authorized.
 - If the participant need changes and service is needed beyond what is currently authorized, a new request will have to be submitted and approved.
- For service plans that were created on or after October 15, 2010, no prior approval was created.
 - In order to create an authorization for any service, that service will have to be requested and approved.
 - This is for an authorization that will be created at the same level, for less, or for more units than currently authorized.
- **When creating a new service plan, the assessor needs to indicate the requested service level for EACH service identified in the service plan.**

- **When reviewing a new service plan, the reviewer needs to enter an approval level for each service prior to signing off of the service plan.**
- Only services identified on the service plan can be requested and approved.
- Only services that have been approved can be referred to a provider and authorized.
- Services can only be referred for the current approval level or less.
- Services can only be authorized, ongoing, for the current approval level or less.
- If there is an open authorization, the case manager can make a referral to a new provider. The authorization cannot be completed for the new provider until the current provider is terminated.
- If more than one provider is providing any service, then the total authorized units of both providers must be within approved limits. For example, if two providers are authorized to provide, personal care II and the approved limit is 15 hours per week, then the two authorizations combined can add up to no more than 15 hours per week total.
- Temporary increase in services
 - The case manager can do an authorization for up to 10 units above the current approved level for an authorization that is not longer than one week. The authorization must have a start and end date. For example, if the approved level for Personal Care II is 10 hours per week, the case manager can authorize 20 hours of Personal Care II as long as the end date is on the authorization and is no more than seven days after the start date. This is being done so short term, temporary increases do not have to be approved.
 - If an increase is needed for longer than one week for units greater than the current approved level, a new service level must be approved.
- If the reviewer denies a service request, zero (0) needs to be entered for the approval level.
 - Services that have been approved at a service level of 0 cannot be referred or authorized to providers.
- If a service is requested, but has not been reviewed and approved and if the service plan is amended to no longer include that service, the service will be removed from the prior approval screen. However, if the service has been approved, it will stay on the prior approval screen.
- There is a new nurse role – “State Nurse and Service Plan Approver”. Nurses in your office who will approve service levels need to have their role changed.
- The following people in the CLTC offices are allowed to approve service levels: area administrator, lead team nurse, lead team case manager, case manager level 2, and nurse consultants with approval authority.
- People who have the right to approve services will have a prior approval tab on their dashboard indicating participants with approval requests.
- Once all services are approved, the participant will be removed from everyone’s dashboard who has approval authority.
- If some but not all services are approved or denied, the notification will remain on the dashboard of approvers until all services have been approved or denied.
- When a service(s) has been approved or denied, a notification will be on the case manager’s dashboard for 14 days.

Guidelines For Authorizing services

1. Any service level that would have required Central Office approval in the past will continue to do so.

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- Until phoenix is able to automatically send these to Central Office, you will need to send an e-mail request to Susie Boykin with the CLTC#.
 - The “Rational for Increased Service Units” need to be completed by the case manager and approved by the area administrator prior to contacting Susie.
 - The guidelines for what requires Central Office approval is located in Phoenix in the “Help” section.
2. The policy on incontinence supply authorizations need to be followed.
 3. Consider a mix of PC1/PC2/Agency Companion and/or Attendant/Companion Individual. In many cases the higher cost of hand-on service is not required for the entire time the worker is in the home.
 4. Use the nutritional supplement guideline in making authorizations for this service.
 5. For persons authorized for Adult Day Health Care, attendance at Adult Day Health Care should be considered when authorizing in-home hours.
 6. The “Guidelines For Determining In-Home Hours” should be used in determining total hours of in-home service authorized per week.